

STRATEGIC INFORMATION COMPONENT ANNUAL REPORT 2025

Reporting year: 2026

*Ministry of Health & Wellness
Health Promotion and Protection Branch
52-60 Grenada Crescent
The Reit building*



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INTRODUCTION

Overview

The Strategic Information Component (SIC) plays a pivotal role in the monitoring and evaluation of the HIV response in achieving epidemic control of HIV, as well as the control of Tuberculosis. The Component is also critical in monitoring sexually transmitted illnesses to achieve key programmatic targets and the mother-to-child transmission of HIV and Syphilis, to maintain elimination status from the World Health Organization.

The SIC is guided by the National Strategic Plans for HIV and TB, which are used to frame strategic interventions nationally and sub-nationally. The following outlines the key strategies that are outlined in the National Strategic Plan for HIV, specifically under objective SO5, which reads *“to develop and maintain a robust monitoring and evaluation framework that can guide all aspects of the HIV epidemic”*:

HIV

S5.1 Improve organizational structure and human capacity and review and evaluate the Human Capacity Development Plan for Monitoring and Evaluation Implementation.

This includes monitoring and evaluation training, development and implementation of M&E training modules, updating of job descriptions for M&E Officers and the establishment of a national database of trainees and trainers in M&E.

S5.2 Improve supportive supervision, data audits and data quality, create an audit framework for technical and administrative processes, establish a data auditing team at the national level and conduct biannual audits of sites and the private sector.

This includes the revision and implementation of a supportive supervision protocol and framework, the implementation of a data quality protocol and the conducting of audits.

S5.3 Increase data use at all levels, dedicated M&E Staff for data analysis and interpretation with adequate supervision and accountability and encourage development of parish data bulletins for dissemination nationally.

This includes the generation of reports routinely for feedback and programmatic decisions, increasing the M&E cadre of staff to 75% of the proposed M&E HR framework by 2025, conducting monthly subnational data review meetings and quarterly national data review meetings

S5.4 Improve HIV data management and sharing, integration of HIV data management systems, collaboration with Pharmacy and Laboratory databases, develop a data sharing agreement with MOHW and other MDAs, that ensures timely sharing of HIV relevant data with the national programme.

This includes creating a biannual Epidemic Tracking Report, improved PLHIV monitoring by subnational treatment facilities and the development of a data sharing agreement.

S5.5 Increase communication and culture around M&E

This refers to the implementation of a communication plan.

S5.6 Improve data collection and monitoring

This includes updating of M&E Operational Guidelines, development of Regional M&E Plans and the standardization of data collection tools.

S5.7 Improved research and evaluations

This includes recommissioned MERG, development and implementation of a Research Agenda and evaluation of implemented strategies.

Key Gaps Addressed:

- Lack of supportive supervision and infrequent data audits
- Poor data quality and underutilization at all levels
- Fragmented, standalone HIV databases
- Limited communication around M&E outputs
- Absence of a standardized M&E structure and dedicated personnel
- Lack of a defined research agenda and work plan
- Inconsistent data collection practices

Tuberculosis (TB)

The SIC is guided by the National Strategic Plan for Tuberculosis for monitoring and evaluation activities, specifically Objective 1.3 which states “*Optimize TB data management including surveillance and vital registration*” and Objective 3.1 which states “*Implement operational research to generate evidence on the TB situation and for TB control programme*”.

Objective 1.3: Optimize TB data management including surveillance and vital registration

Operational Activities as per the Strategic Plan:

- 1.3.1 Promote compulsory notification of TB cases by private and public sector providers.
- 1.3.2 Analyze data on epidemiologic, programme management, care provisioning, sociodemographic and the other determinants of health to help identify gaps in TB control, risk factors for, and populations vulnerable to TB.
- 1.3.3 Utilize data from the monitoring and evaluation framework to effect programmatic changes.
- 1.3.4 Promote quality recording of deaths due to TB in the vital registry system.
- 1.3.5 Capacity building of healthcare workers at site levels to improve data quality collection to aid in decision making in planning, implementation, M&E and research.
- 1.3.6 Carry out bi-annual cross-programme data quality assessment (DQA) between TB and HIV programmes.

Objective 3. 1: Implement operational research to generate evidence on the TB situation and for the TB control programme.

Operational Activities as per the Strategic Plan:

- 3.1.1 Participate in international research for the development of new technologies.
- 3.1.2 Align TB operational research with the National Health Research agenda.
- 3.1.3 Organize meetings, workshops, or seminars to create national TB research networks that enable NTP to liaise with other key units/areas within the MOHW, or other state entities.
- 3.1.4 Compile an inventory of all TB research carried out and its effect or impact, which would permit new research to be formulated, avoid repetition, and facilitate reassessment of activities based on the results obtained.
- 3.1.5 Develop an operational research agenda to understand factors unique to TB among vulnerable groups.

Sexually Transmitted Infections (STIs)

The SIC is guided by the National Strategic Plan (NSP) for STIs for monitoring activities, specifically Objective SO2 which reads “*Enhance surveillance, data systems and research*”

Operational activities under this strategic objectives are as follows:

- Strengthen routine surveillance systems (DHIS2)
- Establish and expand sentinel surveillance sites
- Scale up antimicrobial resistance monitoring
- Conduct quarterly and annual analyses of reported STI cases
- Undertake operations research

OPERATIONAL PLAN IMPLEMENTATION FOR THE FISCAL YEAR 2025/2026

Table 1: 2025/2026 Operational Plan Achievement Status

Activity	Performance Measure	Status	Challenges	Reasons for Non-Achievement
Development of the TB Data Quality Audit tool	TB data quality audit tool developed by Q4	Achieved		
Development of the STI Data Quality tool	STI data quality audit tool by Q4	Achieved		
HIV Data Quality Assessment	100% of planned audits completed by Q2	Achieved		
	HIV audit reports and action plan produced by Q3	Achieved		
	Executive summary and audit by Q3	Achieved		
Supportive Supervision Visits	Quarterly Supportive Supervision Visit report produced 2 weeks after site visit	Achieved		
Quarterly Performance Review	QPR reports to be submitted 2 weeks after the last meeting for the quarter	Achieved		
PMTCT Quarterly Data Review Meeting with RHAs	3 data review meeting reports produced by Q3 Dataset finalized by Q4	Achieved		
M&E Technical Working Group Meeting	4 M&E meeting reports submitted by Q4	Achieved		
Conduct training on the use of Microsoft Power Business Intelligence (Power BI) with national M&E and Strategic Information Officers to enable data visualization	Report produced on Power BI training by Q3	Achieved		

Activity	Performance Measure	Status	Challenges	Reasons for Non-Achievement
and the use of GIS technology to identify hot spots for HIV new infections and prevalence among key populations.				
Produce Global AIDS Monitoring (GAM) Report	Draft GAM & NCPI Report for 2024 submitted to MPD by Q1	Achieved		
SPECTRUM Report	SPECTRUM Report submitted to UNAIDS by Q4	Achieved		
Monitoring the HIV treatment cascade - Routine data gap	4 HIV Cascade reports submitted by Q4	Achieved		
Monitoring of the STI Database	4 STI Database Reports submitted by Q4	Achieved		
Monitoring of the PMTCT Database	4 PMTCT Database Reports submitted by Q4	Achieved		
Monitoring of the HIV Prevention Database	4 HIV Prevention Database reports submitted by Q4	Achieved		
Monitoring of the TB Database	4 TB Database Report submitted by Q4	Achieved		
Operational Research Report	2 operational research submitted by Q4	Achieved		
Management of Data Repository for strategic information	Data Repository printed by Q3	Achieved		
Review of existing STI database to improve efficiency	STI database updated by Q3	Achieved		
Review of existing PMTCT database to improve efficiency	PMTCT database updated by Q3	Achieved		

Activity	Performance Measure	Status	Challenges	Reasons for Non-Achievement
Development of technical guidance for DHIS 2 database	SOP Manual for DHIS 2 developed by Q2	Achieved		

STRATEGIC OBJECTIVES UPDATES FOR HIV

S5.1: Improve organizational structure and human capacity. Review and evaluate the Human Capacity Development Plan for M&E.

The table below provides a breakdown of the staff complement in the Strategic Information Component:

Table 2: Staff breakdown in the Strategic Information Component

Job Title	Temporary (Y/N)	Appointed (Y/N)	Remarks
Database Manager	N	N	On contract against a post
HIV Database, System Administrator	N	Y	
Monitoring & Evaluation Officer	N	Y	
Monitoring & Evaluation Officer	N	Y	
Monitoring & Evaluation Officer	N	Y	
Monitoring & Evaluation Officer	Y	N	Employed as a consultant. The creation of a post is being pursued.
Monitoring & Evaluation Officer (Results Based Contracting)	Y	N	Employed as a consultant.
Programme Administrator	N	Y	
Data Entry Clerk	Y	N	
HIV/STI Information Officer	N	Y	
Programme Coordinator – Strategic Information	N	Y	Programme Administrator

S5.2: Improve supportive supervision, data audits and data quality, create an audit framework for technical and administrative processes, establish a data auditing team at the national level and conduct biannual audits of sites and the private sector.

Data Quality Audits

The Strategic Information Component of the HIV/STI/TB Unit conducts annual data quality audits to ensure that the data used for decision-making is accurate, complete, consistent, timely, and reliable across all levels of the programme.

In 2024, the national overall Data Quality Audit (DQA) score (average) was 85%. Consequently, sites that scored 85% or below were selected for inclusion in the 2025 Corrective Data Quality Audit conducted between May 8 and July 11, 2025.

A total of 1,317 medical records were reviewed and audited during this period. Twelve (12) indicators were examined for historical HIV care information, demographic information, retention in care and treatment information, viral load information and CD4 information (CD4 date and results were first formally audited during this audit; however, it did not contribute towards the overall score). Additionally, the organization, data collection, preparation, and storage of reports were assessed through interviews with the site staff.

The 2025 Corrective Data Quality Audit results indicate a significant decline in overall data quality performance compared to 2024. The corrective DQA score decreased to 68%, reflecting an 8% reduction from 76% in 2024, although the performance remained within the “Acceptable” compliance category. Despite this classification, the decline signals systemic challenges in maintaining accurate, complete, and secure patient data.

The largest decline occurred in the preparation and secure storage of reports, indicating increased risk to patient confidentiality and weaker compliance with the Data Protection Act. This decline is due to inconsistent password protection of electronic files and inadequate safeguards for sensitive patient information, making data governance the most urgent area requiring intervention.

Closely linked to this is the decline in accuracy of high-impact clinical and programmatic indicators, particularly: Next appointment dates, Viral load dates and results, Last clinic visit dates, and ARV treatment dates. These inaccuracies directly affect patient tracking, retention in care, and treatment monitoring, posing risks to both clinical management and programme reporting.

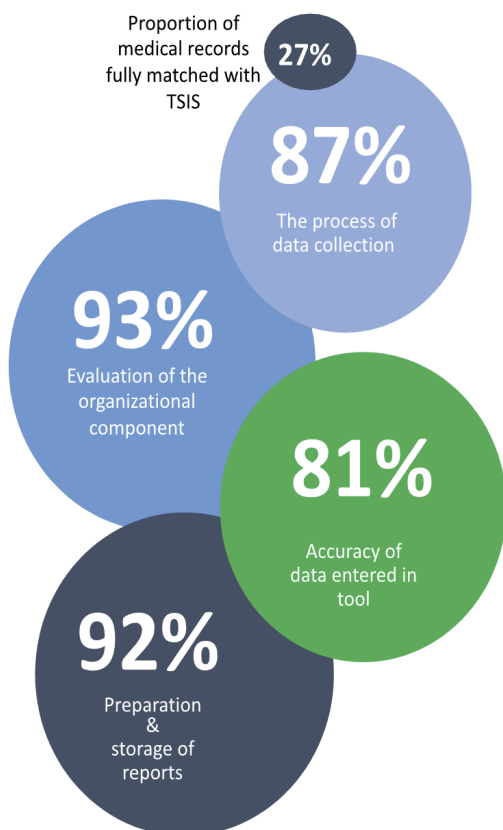
Additionally, the lowest score in the proportion of fully matched medical records reflects broader systemic issues in record reconciliation between paper files and TSIS, further compounding data reliability concerns.

Performance across entities remained uneven, with only half of the audited entities performing above the national average. Independent and civil society sites continued to outperform regional health authorities, suggesting differences in data management practices and resource capacity.

Overall, the audit underscores an urgent need to strengthen data accuracy for key clinical indicators and significantly improve data protection and reporting practices. Without targeted corrective actions in these areas, declines in data quality may continue to undermine programme performance monitoring and clinical decision-making.

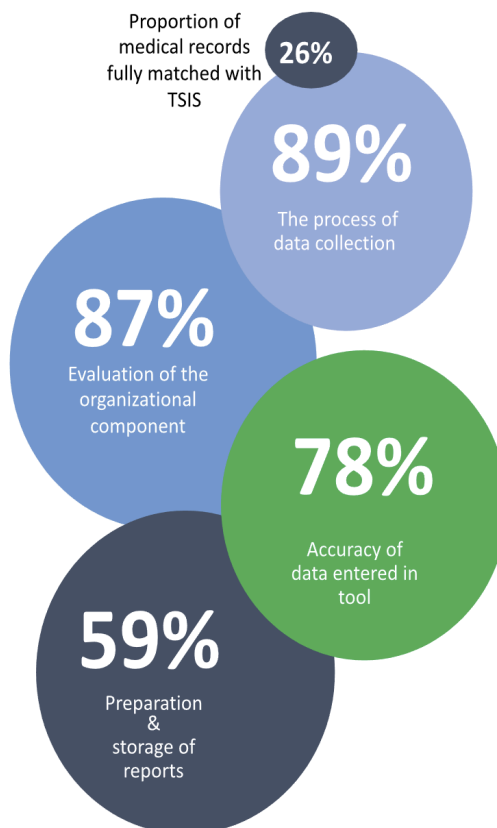
2024 Audit

Average 76%



2025 Audit

Average 68%



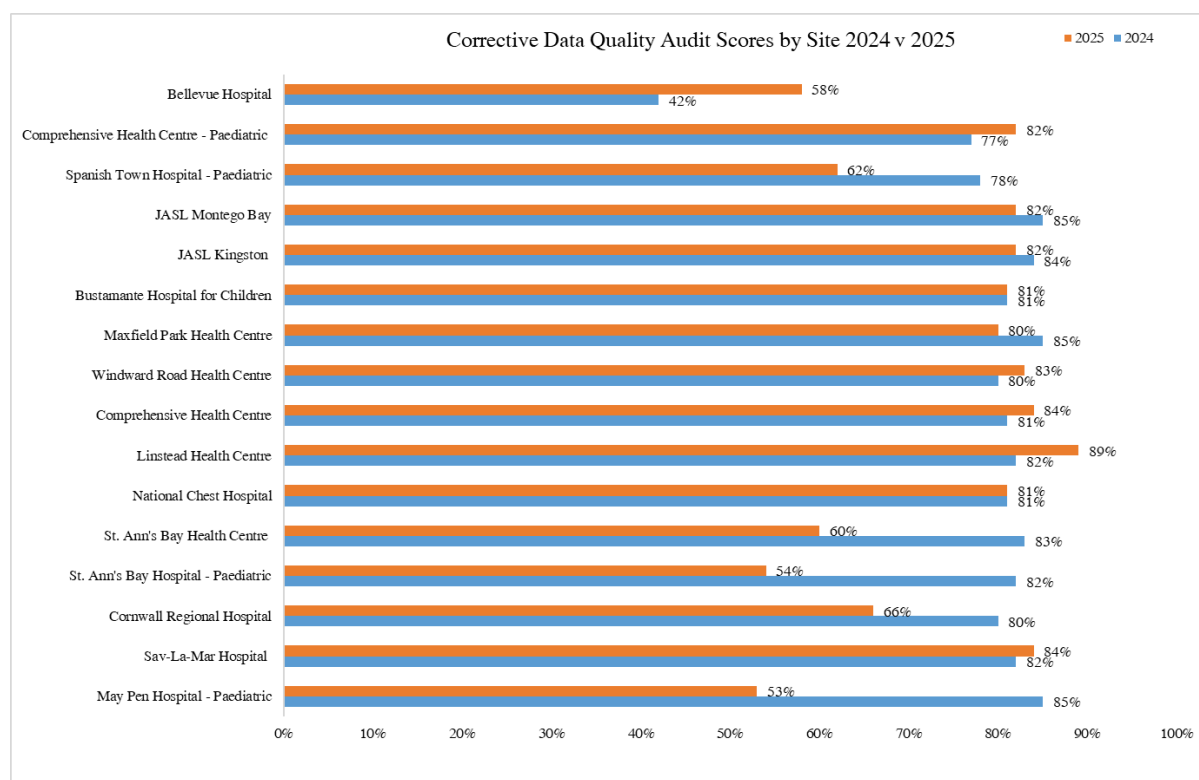


Figure 2: Compliance Scores by Site 2024 v 2025

Figure 2 shows the site-level results, which demonstrate a general decline in data quality performance from 2024 to 2025, with most facilities recording lower scores in 2025. The most pronounced decline was observed at paediatric and hospital-based sites. May Pen Hospital – Paediatric, St. Ann’s Bay Hospital – Paediatric, and St. Ann’s Bay Health Centre experienced steep drops. Spanish Town Hospital – Paediatric also recorded a notable decline.

Some sites showed little improvement, though performance remained suboptimal. Bellevue Hospital improved from a very low baseline but continued to be the lowest-performing site overall, indicating persistent structural and organizational data quality issues.

Independent and civil society sites, particularly the Jamaica AIDS Support for Life (JASL) locations, remained among the stronger performers, although both Kingston and Montego Bay sites experienced slight declines and fell below the 85% threshold. Linstead Health Centre stood out as one of the better-performing sites in 2025.

Overall, the site-level scores highlight uneven performance and widening gaps between facilities, with the sharpest declines occurring in paediatric and hospital settings. These findings emphasize the need for targeted, site-specific corrective actions, particularly focused on strengthening clinical indicator accuracy, record reconciliation, and routine data quality monitoring.

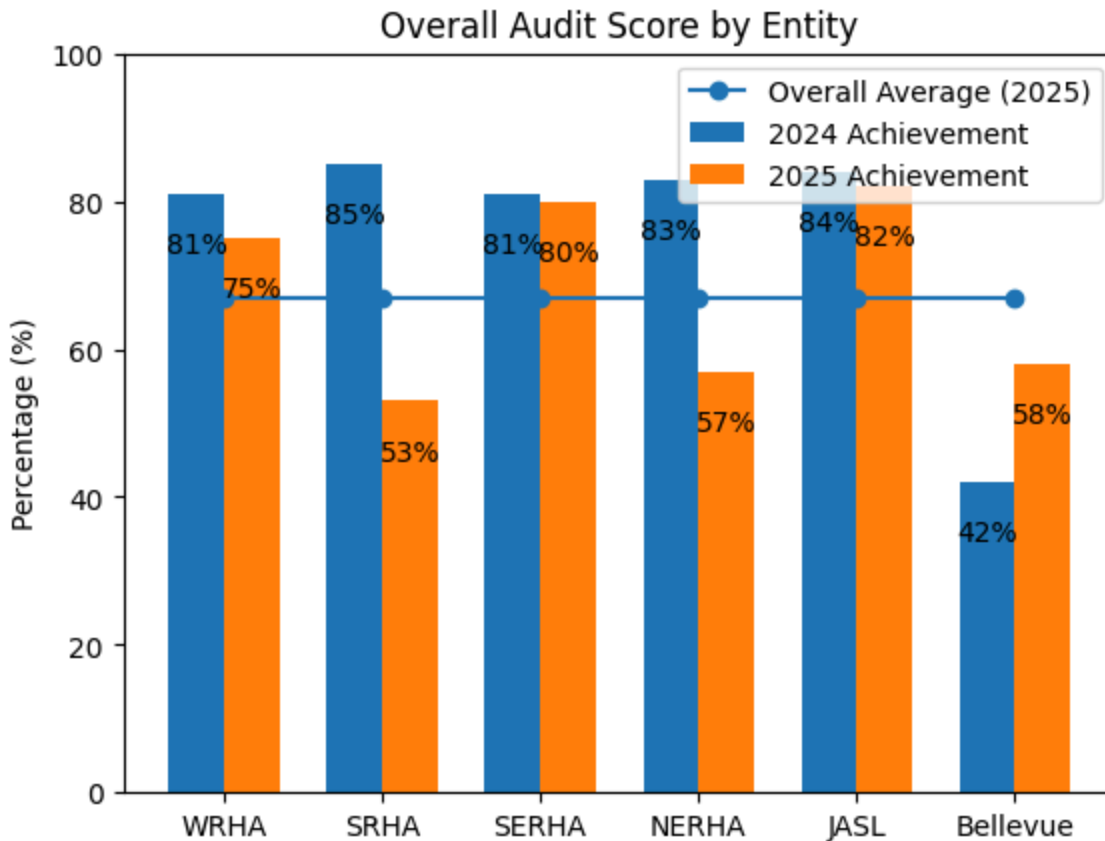


Figure 3: Overall DQA Score by Entity, 2024 v 2025

Figure 3 highlights that three of the six entities audited had an overall DQA score above the overall average of 68%. Seventeen per cent (1/6) of the entities achieved a “Good” score on the DQA compliance scale. The highest performer was JASL (82%), and the lowest was Bellevue (58%).

Overall data matching declined from 81% in 2024 to 76% in 2025, indicating persistent data quality gaps (Figure 4).

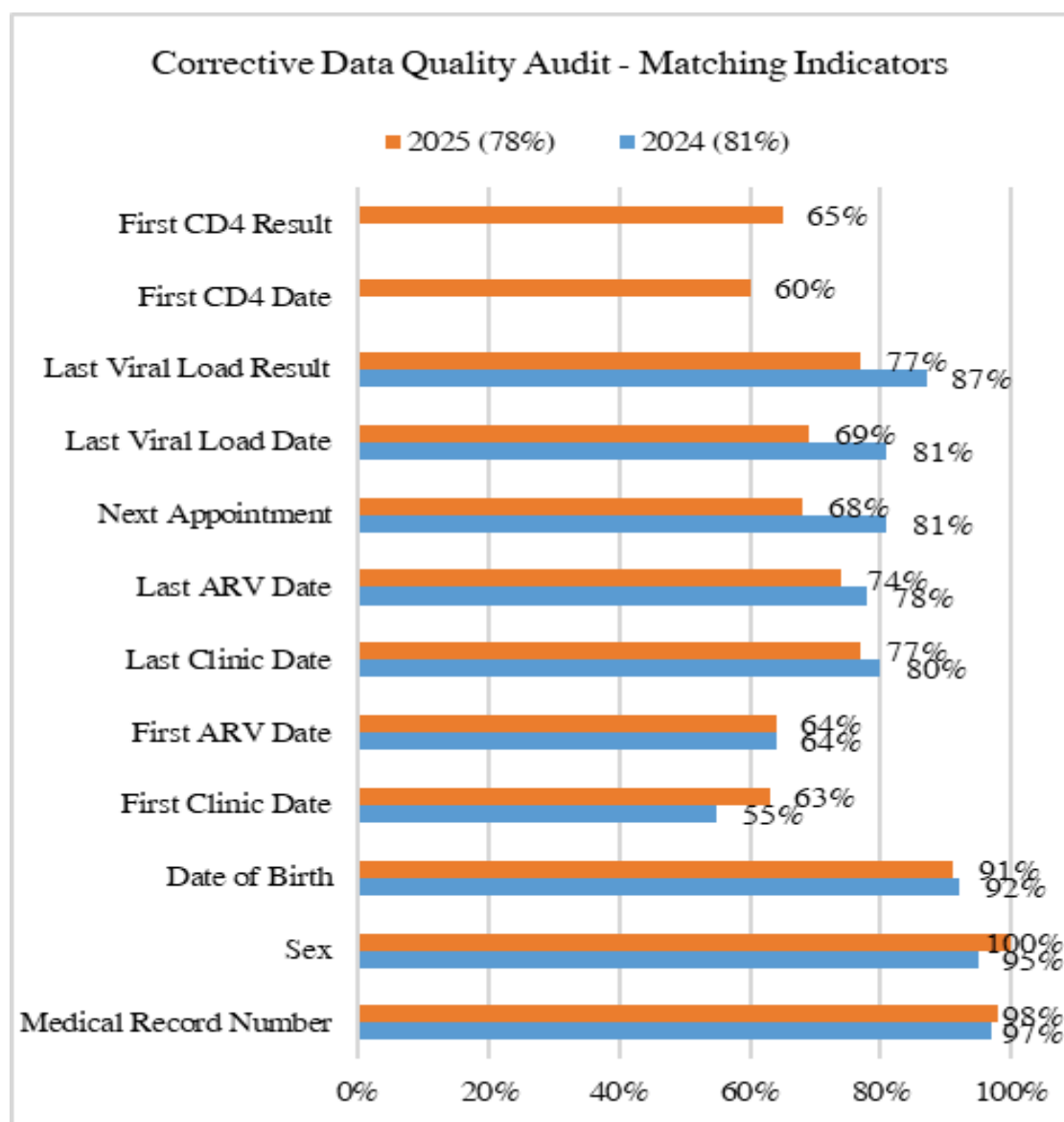


Figure 4: Overall Corrective Data Quality Audit - Matching Indicators (2024 v 2025)

Key strengths in data matching

- Sex remained high and improved to 100% in 2025 (from 95%).
- Medical Record Number stayed consistently strong (98% in 2025, 97% in 2024).
- Date of Birth remained high with only a slight dip (91% in 2025, 92% in 2024).

Areas with declines

- Last Viral Load Result dropped from 87% to 77%.
- Last Viral Load Date decreased from 81% to 69%.
- Next Appointment fell from 81% to 68%.
- Last ARV Date declined from 78% to 74%.

- Last Clinic Date slightly reduced from 80% to 77%.

Persistently weak indicators

- First ARV Date stayed unchanged at 64%.
- First Clinic Date improved to 63% but remains suboptimal.

The First CD4 date and result were first audited during the 2025 Corrective Data Quality Audit; therefore, these indicators were not included in the overall score. However, they received suboptimal scores of 60% and 65%, respectively, indicating the need for improvement.

Key Areas for Action

- Addressing historical indicator inaccuracies.
- Tracking records that need to be updated on a clinic day to reduce missed entries.
- Formalizing transfers on TSIS 2.0 for clients accessing care at other sites.
- Addressing all viral load and CD4 date and result inconsistencies.
- Improvement in the data protection of patient electronic records.

Prevention Data Quality 2025

The 2025 National Prevention Data Quality Assessment (DQA) Audit took place between March and April 2025 across the four Regional Health Authorities (RHAs) and one Civil Society Organization (CSO), Jamaica Aids Support for Life (JASL). This audit evaluated ten indicators that contribute to the generation of Unique Identify Codes (UIC) as well as outreach and demographic information.

In addition, the audit assessed the organization, data collection, preparation, and storage of reports through interviews with site staff. The national target for the 2025 DQA score is set at 95%, allowing for a margin of error of 5%. The overall performance from the 2025 DQA audit resulted in none of the implementing entities meeting the target. North East Regional Health Authority (NERHA) had the highest performance of 88% followed by Southeast East Regional Health (SERHA) 87%, JASL 85%. The lowest performers were Southern Regional Health Authority (SRHA) at 83% and Western Regional Health Authority with 73%.

Table : National Prevention Data Quality Assessment Audit Scoring for 2025

Entity	National Target	2025 Achievement	Ranking
NERHA	95%	88%	Good

SERHA	95%	87%	Good
JASL	95%	85%	Good
SRHA	95%	83%	Needs Improvement
WRHA	95%	73%	Needs improvement

Common errors identified on a monthly basis are incorrect event date examples, dates that are in the future. Late report that affects previous reports. PrEP Incomplete entries, for example, missing documentation of medical eligibility or linkage and initiation dates. Moving forward, monthly data quality checks will be conducted through the Payment for Results process, followed by the annual audit that will be done by the Strategic Information Component.

Supportive Supervision

Supportive supervision was provided for all Regional Health Authorities as well as the University Hospital of the West Indies Infectious Diseases Paediatric Clinic. The visits were centred around the data quality audit corrective actions, as well as other gaps that were noted from routine database monitoring activities. The cascades below are a testament to the improvement noted in data quality, that can be attributed to supportive supervision interventions, among others.

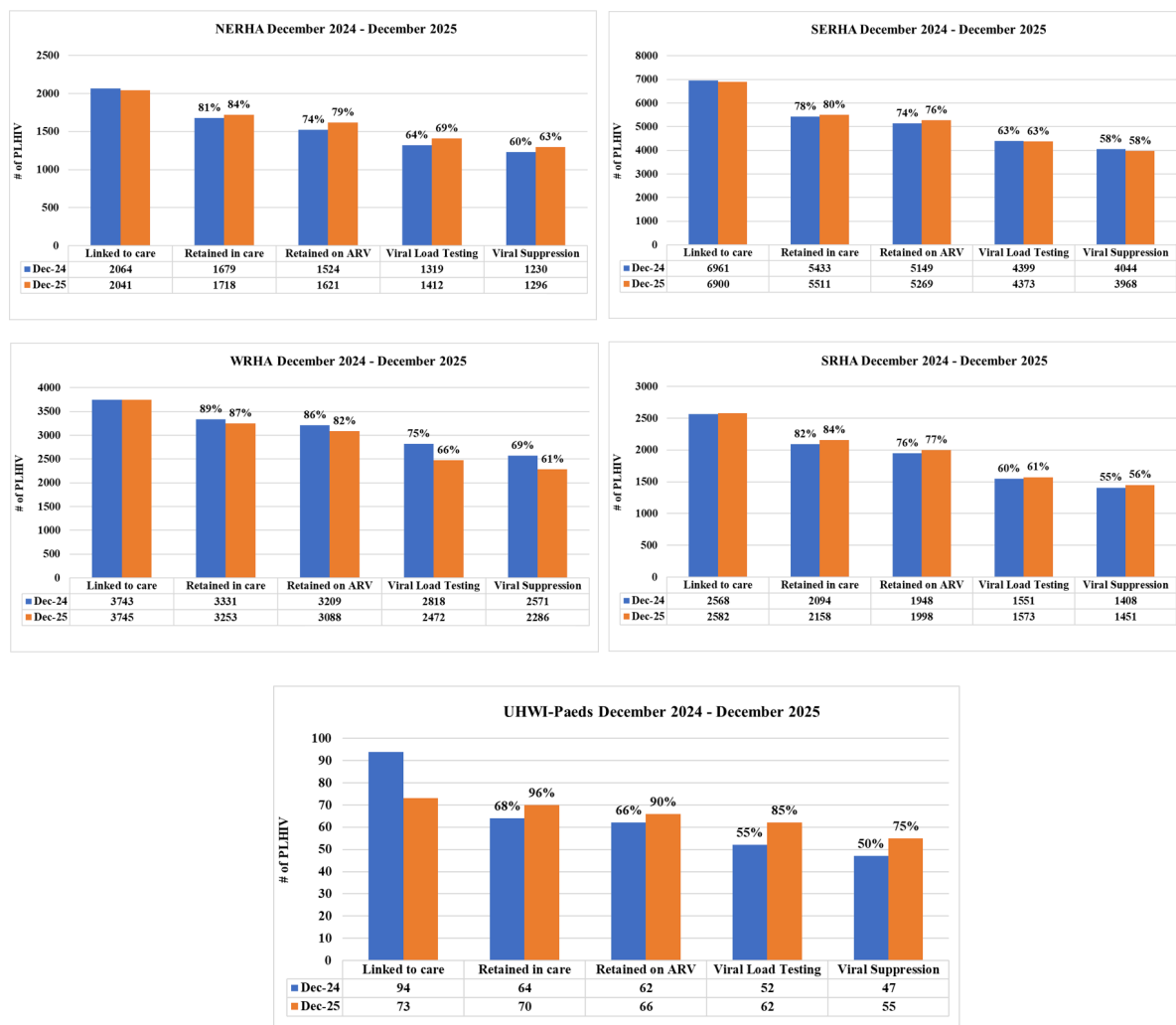


Figure 2: Treatment cascades for the RHA and UHWI Paeds for the period December 2024 to 2025

Improvements were seen in all cascades (Figure 2). Among the gaps addressed were; the timely updating of TSIS, correcting demographic information entered in the database, actively closing cases based on case archival criteria.

S5.3: Increase data use at all levels.

National meetings/workshops

National and sub-national reviews/meetings were conducted throughout the year, bringing key stakeholders together to share new information as well as to gain insights into challenges and best practices in service delivery. The major review/meetings conducted during the year are outlined below, along with the key outcomes and recommendations (Table 4).

Table 4: Annual Review/Meetings.

Type of Meeting	Objectives	Key Outcomes	Recommendations
PMTCT Data Verification Workshop (Feb 2025)	To ensure the completeness and accuracy of reported data, which is critical to national monitoring and by extension the maintenance of the EMTCT status	Key indicators for GAM and Spectrum clarified The dataset for 2025 for all public sites for GAM and Spectrum was completed and entered on the respective platforms	1. Proper documentation for Maternal Syphilis screening and treatment 2. Standardize trimester-specific retesting records 3. Strengthening communication and data exchange between Primary and Secondary Healthcare Facilities 4. Enhance surveillance and HIV mortality reporting
TSIS Data Quality Meeting	To identify data quality gaps affecting the HIV treatment cascade, develop action to correct and implement.	Improvement in the HIV treatment cascade pillars	1. Prescription end date corrections in order to improve the retained on ARV column of the cascade. 2. Updating of viral load results in order to improve the viral load testing and viral suppression columns of the cascade. 3. Increase SMS Uptake in order to improve retention in care and retained on ARV columns.

S5.4: Improve HIV data management and sharing.

Health Information Systems

Treatment Site Information System (TSIS)

TSIS continues to be the database that houses data on all HIV positive persons that have been linked to care, whether at a public or private treatment site.

The estimated number of persons living with HIV (PLHIV) increased from 26,000 to 28,000 as of January 2025, based on the UNAIDS Spectrum process produced using models and software tools and was informed by HIV surveillance data, surveys, and programmatic data, producing historical trends and projections of critical indicators (Figure 1 & 2).

As of January 2025, the number of PLHIV diagnosed increased from 23,500 to 26,500 based on data obtained from the National Surveillance Unit, which increased the percentage of PLHIV diagnosed from 90% in December 2024 to 95% in March 2025 (Figure 1 & 2). As a result of this increase in the diagnosed pillar, retained on ARVs declined.

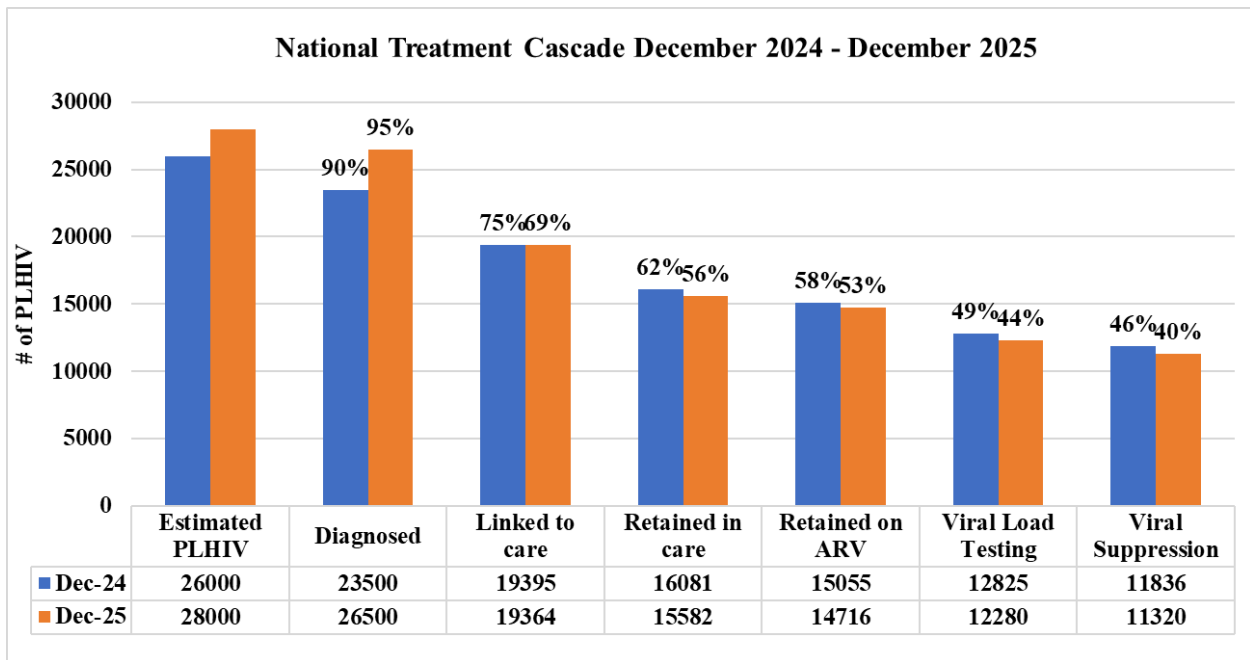


Figure 3: The National Treatment Cascade 2024 vs 2025

The HIV Treatment Cascade (Figure 13) highlights critical stages in the continuum of care: diagnosis, linkage to care, retention in care, retention on antiretroviral therapy (ART), viral load testing and viral suppression. At each step, gaps emerge that weaken the overall effectiveness of the HIV response. One of the most significant gaps occurs after treatment initiation, where individuals are lost to follow-up, not retained on ART, and not virally suppressed. These disruptions can lead to increased morbidity, the development of drug resistance, and ongoing transmission within communities. Addressing these gaps, especially around retention and re-engagement, is essential to improving treatment outcomes and achieving epidemic control.

Table 5: SI led activities attributable to the changes in the cascade

Indicator	Activities linked to change	Percentage change and (Difference in actual numbers) 2024 compared to 2025
Linked to care	1. The estimated PLHIV were adjusted based on the 2025 Spectrum country estimates. 2. Data cleaning which consists of the removal of patients that have died, migrated or have been out of care for 10 or more years.	6% decline (declined by 31)
Lost to follow-up (linked to care - retained in care)/linked to care	1. Hurricane Melissa distributed LTFU initiatives and displaced patients in WRHA and SRHA. 2. The shutting down of HCJ due to the cutting of funds from USAIDS resulted in lack of data entry from private physicians and the displacement of patients.	3% increase (increased by 468)
Retained in care	1. The estimated PLHIV were adjusted based on the 2025 Spectrum country estimates. 2. Lack of timely data entry. 3. The shutting down of HCJ due to the cutting of funds from USAIDS resulted in lack of data entry from private physicians and the displacement of patients.	6% decline (declined by 499)
Retained on ARV	1. The estimated PLHIV were adjusted based on the 2025 Spectrum country estimates. 2. Lack of timely data entry. 3. The shutting down of HCJ due to the cutting of funds from USAIDS resulted in lack of data entry from private physicians and the displacement of patients. 4. Patients who have missed their appointments. 5. Prescription end date error.	5% decline (declined by 339)
Viral Load Testing	1. The estimated PLHIV were adjusted based on the 2025 Spectrum country estimates. 2. The slow processing of viral load by NPHL. 3. Lack of timely data entry.	5% decline (declined by 545)
Viral Suppression	1. The estimated PLHIV were adjusted based on the 2025 Spectrum country estimates.	6% decline (declined by 516)

Table 6 highlights the data cleaning activities in TSIS which include deletions, transfers, mergers, out of care and deaths, each contributing to improved data accuracy. Please see below for the definition of key terms:

- Deletions involve the removal of duplicate entries and the consolidation of merged records.
- Transfers refer to clients being reassigned from one site to another, based on field requests.
- Deaths represent individuals with HIV who have passed away; the reported figures reflect the date their status was updated in TSIS, which may not correspond to the actual date of death.
- Archived cases must meet the following criteria:
 - 5 years from their last HIV clinic visit for patients 13 years from the recorded date of HIV diagnosis; or 13 years from the date first seen at a HIV treatment site for patients without a date of diagnosis.

OR

(add bullet) 5 years from their last HIV clinic visit for clients with clinical diagnosis of Advanced HIV or AIDS (CD4 ever < 350).

The Medical Officer of Health must review and approve the case(s) to be archived before it is actioned by the Strategic Information Component.

Table 6: TSIS Database cleaning activities

Action	# of PLHIV
Mergers	266
Client Deletions	361
Case archival	171
Transfers	534
Deaths (recorded)	165

District Health Information System (DHIS2)

The District Health Information System (DHIS2) database continues to provide an overview of individuals accessing HIV prevention services. The system aims to facilitate the collection, management, and analysis of data critical for monitoring service delivery, assessing outreach effectiveness, and informing programme decisions. HIV prevention services encompass various interventions such as, reach (educational interaction), HIV counselling and testing, condom distribution, and pre-exposure prophylaxis (PrEP). Tracking the uptake and outcomes of these services helps the programme to assess coverage, identify gaps, and tailor interventions to populations at higher risk.

For the year 2025, the HIV Programme interventions successfully reached a total of 152,680 individuals across targeted populations, with the highest engagement seen in the sexually active females (SAF), followed by sexually active males (SAM). Key populations, female sex workers (FSW), men who have sex with men (MSM) and Transgender (TG), contributed 11,421, 5,781 and 482, respectively, accounting for 12% of the overall reach total. All individuals participated in risk reduction conversations, which served as a major strategy for education and prevention, reaching all groups extensively.

Persons identified with high-risk behaviours, occurred in 9,447 instances highlighting ongoing challenges in risk engagement among these populations. Referrals to additional health services were made in 3,593 instances, facilitating access to comprehensive care.

HIV screening efforts were also significant, with 136,314 people tested, accounting for 89% of the total number reached. The SAF population again had the highest testing numbers, followed by At-Risk-Youth (ARY) and SAM. Distribution of HIV self-test kits was 7,234 across all populations; the lowest distribution continues to be among MSM and TG.

A total of 899,019 pieces of condoms were distributed, with the highest distribution among SAF and SAM populations, indicating a focus on prevention tools. Pre-exposure prophylaxis (PrEP) was provided to 609 individuals, mainly within the broader at-risk populations.

Table : Total Entries in DHIS2 by Service Delivery and Populations January to December 2025

Intervention	ARY	F50+	M50+	SAF	SAM	FSW	MSM	TG	Total
Risk Reduction Conversation	34,032	9,891	8,708	50,722	31,643	11,421	5,781	482	152,680
High Risk Behaviour identified*	1,837	259	370	1,515	2,145	2,523	731	67	9,447
Reach	34,032	9,891	8,708	50,722	31,643	11,421	5,781	482	152,680

Testing	30,654	8,794	7,414	46,175	26,689	10,713	5,454	451	136,314
HIV/ST	1,379	348	394	2,496	2,357	172	82	6	7,234
Condom Distribution	175,999	41,772	41,465	274,679	172,282	158,906	2,9740	4,176	899,019
Referrals of services	435	380	240	608	540	527	802	61	3,593
PrEP	60	3	0	11	169	108	250	8	609

The DHIS 2 database was updated to capture PrEP and other health services. These services include HIV positive referrals and referred site, syphilis referrals and referred site, psychological referral and referred site, STIs referrals and referred site, social services referral and referred site and PrEP referral and referred site. A new section for PrEP initiation was also added. This section captures general information on sexual practices within a year, drug and contraceptive screening, PrEP history, PrEP linkage and PrEP Initiation and retention.

Enabling Environment and Human Rights (EEHR)

During the reporting period, a consultancy was initiated to update and enhance the existing Enabling Environment and Human Rights (EEHR) Online Dashboard. The EEHR Dashboard was developed to support strategic monitoring and reporting, however, the need for system upgrades became evident to ensure more effective tracking, measurement, and reporting of progress in addressing issues faced by key and vulnerable populations. Furthermore, strengthening this digital platform is essential to advancing the implementation of the EEHR Human Rights Work Plan and improving data-driven decision-making within the HIV, STI, and TB response. This update to the EEHR Dashboard is expected to significantly strengthen monitoring and evaluation efforts, improve information accessibility, and promote greater transparency and accountability in supporting human rights-based health programming.

EEHR Dashboard Overview for 2025

The United Nations Development Programme funded a consultancy to update the database. The consultancy commenced with a meeting involving UNDP and key stakeholders led by the Enabling Environment and Human Rights Officer. Implementation of the consultancy was carried out over the period January to September 2025. This process incorporated initial feedback from the team, which had been monitoring the dashboard under the NFPB, particularly regarding challenges experienced in its functionality and use.

The scope of work for the dashboard update included the following:

- Provision of an additional six (6) months of monitoring and maintenance services for the online reporting dashboard.

- Convening of five (5) technical consultative meetings to address user interface and administrative issues experienced by the Dashboard Management Team.
- Population of the dashboard with impact and outcome indicators and corresponding definitions, based on reference sheets from the 2023 EEHR Monitoring, Evaluation and Learning (MEL) Plan provided by the Dashboard Management Team.
- Facilitation of two-day virtual training sessions, following the resolution of technical issues, to strengthen stakeholders' capacity for the efficient and effective use of the EEHR Dashboard.
- Updating of the existing EEHR Dashboard Manual to reflect system enhancements, including but not limited to:
 - o Procedures to ensure successful user login
 - o Steps for creating entity-specific login credentials
 - o Definitions of headings and fields within the Excel data templates
 - o Guidance on populating and editing dashboard components, including goals, indicators, outputs, outcomes, and activities
 - o Addressing additional gaps identified during the upgrade process
- Facilitation of the formal handover of the EEHR Dashboard to the Government of Jamaica.
- Submission of a Handover Report summarizing the development process, key features of the dashboard, and any outstanding issues or recommendations for future improvement.

Following the dashboard update, the consultants conducted training sessions with the HSTU team on backend data management, including uploading and maintaining information. Additionally, training sessions were held with Regional Health Authorities (RHAs) and Civil Society Organisations (CSOs) to reintroduce the dashboard and provide foundational guidance on uploading data related to implemented activities. These sessions also strengthened stakeholders' understanding of how their activities align with and contribute to the overall goals and indicators of the EEHR Programme.

The way forward

At present, the HSTU team is responsible for completing the population of the dashboard with impact and outcome indicators. However, a consultancy to revise the MEL Plan is currently underway, and this process may result in changes to the indicator framework. As such, full population of the dashboard is expected to be completed following the finalization of the MEL Plan revision.

Upon completion of both the MEL Plan consultancy and the dashboard population process, an additional round of training will be conducted with CSOs, RHAs, and other relevant entities. At that stage, entities will be required to actively report into the dashboard. If necessary, iTech will be re-engaged to support further capacity building of the HSTU team to effectively facilitate this process.

STI and Contact Investigators Reports

The following activities were conducted by the SI Component during the period under review for STI and contact investigators reporting databases:

1. Created health centre sites and assigned contact investigators to them to facilitate site level data entry for STI and CI monthly reports.
2. Trained all contact investigators in data entry and report generation.
3. A data review meeting was conducted, during which gaps in reporting compliance were identified, and targeted recommendations were provided to address these issues.
4. The STI Monthly report was revised and updated for implementation in January 2026. The following diseases/syndromes were added 1) Congenital 2 - 4 years, 2) Cervicitis, 3) Chancroid, 4) Granuloma Inguinale, 5) LGV, 6) Hepatitis B/C (New cases) and 7) HTLV I / II (New cases). All stakeholders were retrained in the use of the revised monthly reports.
5. The Contact Investigator Monthly report was revised and updated for implementation in January 2026. The primary addition was PrEP by age group, sex and targeted population indicators. All stakeholders were retrained in the use of the revised monthly reports.

PMTCT Database

Data management activities were conducted including the cleaning of data entered in the PMTCT database, as well as liaising with the field for incomplete entries to be addressed.

The PMTCT Monthly report was revised and updated for implementation in January 2026. The primary additions are 1) GAM indicator set and 2) SPECTRUM indicator set. All stakeholders were retrained in the use of the revised monthly reports.

Server Updates

Through the support of Centres for Disease Control and Prevention, the HSTU acquired a Dell PowerEdge R6525 server to replace aging hardware (old servers), in order to enhance systems performance and reliability. Subsequently to being commissioned by ICTU the following applications were loaded to the server starting January 2025. HealthJam platform, this is a case management platform funded by Caribbean Technical Assistance and Education Center for Health (C-TECH) to support patient adherence and retention through active patient engagement. At the completion of the Enabling Environment and Human Rights (EEHR) consultancy. ITech supported

deployment of the online dashboard to server space provided by this hardware. Other resources currently housed on the server include: Prevention Database; STI, EMTCT and CI monthly reporting summaries all running on the latest version of the DHIS platform.

S5.5: Increase communication and culture around Monitoring & Evaluation.

No activity was conducted under the period of review for this strategic objective. However, the Component intends to develop a communication plan centred around how information will be shared with key stakeholders (e.g. dashboards, bulletins) as well as trouble shooting database related matters and the chain of command to be followed to resolve them.

S5.6: Improve data collection and monitoring.

Development/Updating of Manuals

A DHIS 2 manual was developed to provide guidance on the usage of DHIS 2, including steps to troubleshoot common issues that may arise. The next step is for the manual to be printed and distributed to key personnel.

Capacity Building

Four (4) major capacity building activities were conducted throughout the period under review:

Power BI Training

A qualified consulting firm was engaged to deliver targeted Power BI training to Monitoring & Evaluation Officers and Data Managers, with the aim of strengthening operational research and enhancing routine data analysis and reporting to inform policy development, programme implementation, monitoring and evaluation, and performance management. A total of seven training sessions were conducted across three groups: national, regional, and civil society organizations between August and October 2025, specifically on August 14–16, September 22–23, and October 1–3, 2025.

The training reached 40 participants from the HIV/STI/TB Unit, the four Regional Health Authorities, and key civil society partners, including Jamaica AIDS Support for Life (JASL), Children First Agency, The ASHE Company, and C.H.A.R.E.S. Each participant was equipped with

Power BI software procured by the Unit, along with a printed handbook to support learning during and after the sessions. The consultant incorporated real programme data to guide the development of customized dashboards, enabling participants to apply skills in a practical, job-relevant context. Pre- and post-training surveys were administered to assess user experience, demonstrating notable improvements in participants' confidence and understanding of Power BI and its application in data analysis and reporting.

Operational Research Writing Workshop

A two-day Operational Research Writing Workshop was held at the AC Marriott Hotel, Kingston, on July 24–25, 2025. The workshop successfully facilitated development of research topics and established timelines for poster development with the support of technical advisors. The primary objective of the workshop was creating three posters designated for presentation at the Treatment, Care and Support Forum, scheduled for September 10–12, 2025. The workshop and its preparatory activities ensured effective coordination, technical engagement, and readiness for the upcoming events.

These activities were aligned with the National Strategic Plan (NSP) for HIV 2023–2030, which prioritizes governance, prevention, treatment, enabling environments and human rights, and strategic information. The agenda focuses on aligning resources with national needs, strengthening research capacity, generating evidence to inform policy and programming, and ensuring systematic monitoring and dissemination. Capacity building efforts included SPSS and Power BI training for Regional Health Authorities (RHAs) and Civil Society Organizations (CSOs), an operational research writing workshop, and post-training technical support. Research outputs were expanded through the national research repository, which now includes capacity-building materials, and the publication of the first volume of *Unveiling Hope*, a research booklet.

Overall, the workshop not only facilitated progress on research being developed but also established structured timelines for completion and dissemination. With clear submission deadlines and research topics assigned to upcoming forums, the research teams were well positioned to share their findings in national and regional platforms later in the year. Three researches were successfully completed and submitted by the deadline for the TCS Forum, surpassing the original target of producing just two posters by 150%. All three research posters were presented at the Annual Treatment Care and Support Forum held in September 2025. See three abstracts in the Appendix for operational research conducted during the period of review.

Treatment Site Information System (TSIS 2.0) Trainings

TSIS 2 and TSIS Report training sessions were conducted on July 30, 2025, with the North East Regional Health Authority (NERHA) at the St. Ann Health Department, and on October 2, 2025, at Bellevue Hospital within the Infectious Disease Unit, reaching a total of 34 staff members. At NERHA, 30 newly onboarded multidisciplinary staff including Medical Doctors, Treatment Care and Support Officers, Adherence Counsellors, and Nutritionists participated, while 4 Infectious Disease Nurses were trained at the Bellevue Hospital. Each five-hour session aimed to strengthen staff capacity to effectively use TSIS 2 and its reporting features for programme monitoring and the management of persons living with HIV.

The training covered key areas such as an overview of the TSIS database and its data entry fields, procedures for updating client status (including archiving due to death or migration, deduplication, and transfers), and the use of TSIS reports for data management and analysis. Participants were also guided through building treatment cascades, using pivot tables, and identifying and addressing common data quality issues. By the end of the sessions, participants demonstrated improved understanding and confidence in using TSIS 2 and TSIS reports to support routine monitoring and decision-making.

Table 7: TSIS Trainings to Improve Data Quality

Sites	Activities	Remarks
Bellevue Hospital, UHWI Paeds, NERHA	TSIS2 and TSIS reports training	New staff members were trained to use TSIS2 and TSIS reports to aid in timely data entry and the monitoring of site level data.

DHIS2 Sensitization on Data Validation and Error Resolution

Sensitization sessions on Data Validation and Error Resolution in DHIS2 were conducted across all four Regional Health Authorities between August and October 2025 to strengthen the quality

and reliability of HIV prevention data. Facilitated by the DHIS2 Database Administrator, the sessions targeted frontline data users, including Community Peer Educators, Behaviour Change Communication Coordinators, and representatives from Civil Society Organizations. A total of over 180 participants were engaged.

The sessions focused on common data entry errors, system responses to errors and warnings, event capture and reporting, and best practices for data validation. Interactive discussions and real-world scenarios were used to demonstrate practical solutions, while quizzes reinforced learning. The sessions improved participants' awareness of data quality issues, strengthened their understanding of DHIS2 functionalities, and emphasized the importance of accurate, timely data entry and validation for effective HIV prevention reporting and decision-making.

S5.7: Improved research and evaluations.

Research Repository

The HIV/STI/TB Research Data Repository was conceptualized in 2023 by the Strategic Information Component with the aim of collecting and collating both published and unpublished studies and reports on a quarterly basis. This initiative is designed to provide a comprehensive and organized collection of HIV, Sexually Transmitted Infections and Tuberculosis research in Jamaica and the wider Caribbean, facilitating easy access and navigation for stakeholders involved in the fight against infectious diseases. The repository currently houses over 200 studies in 2025, meticulously organized across 13 distinct areas, with a keen focus on high-priority and priority areas for the HIV/STI/TB Unit. All documents are now accessible through a virtual platform managed by the Strategic Information Component, ensuring that users can efficiently access and utilize the wealth of information available. Updated research was identified for all strategic objectives of the data repository and has been populated. The updated data were initially stored on Google Drive before being transferred to the official website, <https://williamsandremoh.wixsite.com/jmerg>. The necessary steps have been initiated to integrate the data repository into the Ministry of Health and Wellness website.

Operational Research

Four operational research studies were completed during the fiscal year (see abstracts in Appendix). The first study, “Factors Associated with very late presentation among PLHIV in Southern Jamaica Prevalence of HIV Drug Resistance and Its Determinants Among Unsuppressed Persons Living with HIV in Jamaica”, a retrospective cross-sectional study in Jamaica (Oct 2024–Jun 2025), analyzed 67

valid HIV drug resistance samples from unsuppressed PLHIV. Results showed a high prevalence of resistance mutations, especially to NRTIs, with all participants affected, posing risks for treatment failure and fewer regimen options. Lower CD4 counts at ART initiation were significantly associated with more resistance mutations, while viral load and time on ART were not. The findings highlight the need to strengthen lab systems, improve viral load monitoring and timely clinical action, and reinforce early diagnosis, rapid linkage to care, and prompt ART initiation to limit further resistance.

In the second study, “A Risk Assessment Matrix for Identifying Likelihood of Loss to Follow Up in Persons Living with HIV”, a risk assessment scoring matrix was developed to identify HIV patients at risk of becoming lost to follow-up (LTFU) within SRHA treatment sites in Jamaica using data up to June 2024. The tool assigned weighted scores (0–100) to key predictors and was applied to 2,399 eligible patient records from TSIS 2.0, categorizing individuals as retained or LTFU. Findings showed a clear inverse relationship between risk score and retention: patients with low and moderate scores had high retention (100% and 85%, respectively), while those in the critical risk category had the highest LTFU (61%). Statistically significant differences in retention were observed across factors such as age, address type, prior LTFU history, viral load status, and duration in care, whereas gender and linkage time were not significant. Higher risk scores strongly predicted LTFU, supporting the use of the tool to guide targeted interventions, prioritize resource allocation, and strengthen routine patient monitoring, particularly through intensified follow-up for high- and critical-risk individuals.

The third study, “Mortality Patterns and Gaps in HIV Care: Insights from Jamaica, 2021–2023”, analyzed a study of people living with HIV (PLHIV) aged 15 years and older who were diagnosed and enrolled in treatment sites in Jamaica between 2021 and 2023. Factors influencing mortality were examined using Chi-square tests, Kaplan-Meier survival analysis, and Cox regression, with data extracted on July 4, 2025. While mortality outcomes have improved compared to earlier cohorts, they may be underestimated due to 19% of individuals being lost to follow-up. Higher mortality was observed among those aged 60 years and older, individuals with advanced immunosuppression (CD4 <200 or Stage 4), and those without a recorded baseline CD4 count. The link between missing CD4 data and poorer survival highlights potential gaps in testing, documentation, and patient retention. These findings underscore the need for targeted HIV testing, timely initiation of antiretroviral therapy, improved retention in care, and strengthened CD4 testing and documentation practices.

The fourth study was a national analysis titled “Retention in Care Among PLHIV Accessing Care at Public Treatment Sites in Jamaica” which examined HIV care retention among 12,542 people living with HIV using data from the Treatment Site Information System after relevant exclusions were applied. Retention, defined as remaining in care without missing appointments for more than 90 days was 83%, while 17% of individuals were classified as lost to follow-up. Higher retention rates were observed among females and married individuals, whereas lower retention was seen among persons aged 20–44 years and those receiving care outside their parish of residence. The use of SMS reminders was strongly associated with improved retention, and favourable clinical indicators, such as lower baseline CD4 counts and viral load suppression, were also linked to sustained engagement in care. Despite the relatively high overall retention, targeted interventions are needed to better support younger individuals, males, and those accessing care outside their parish, alongside strengthening reminder systems and enhancing service accessibility.

Global AIDS Monitoring Report and National Commitment and Policy Instrument (NCPI)

The Global AIDS Monitoring (GAM) Report and National Commitment and Policy Instrument (NCPI) are frameworks used by national AIDS programmes and partners to assess the state of a country’s HIV response and track progress towards national and global targets. The GAM process continues to serve as a benchmark for international accountability, with lessons from previous rounds guiding improvements in timeliness, quality, and completeness of reporting.

For the 2026 reporting cycle, several updates have been introduced to improve standardization, reduce duplication, and align with evolving guidance. Key indicators such as HIV incidence, AIDS mortality, knowledge of HIV status, viral suppression, and vertical transmission will now be derived directly from final Spectrum files to promote consistency in global aggregation. Prevention indicators have also been strengthened, with a shift from reporting outputs to measuring coverage, evidenced by the introduction of “PrEP need met” and “condom need met,” both incorporating denominators to assess programme reach.

In addition, a simplified bio-behavioural survey (BBS-lite) has been introduced as an alternative data source for key population indicators, enhancing feasibility in data collection. Adjustments have also been made to indicator disaggregations, including the inclusion of transgender populations and streamlined reporting categories. Some data requirements have been removed where availability has

been limited, while additional survey options have been introduced for measuring HIV-related discrimination in healthcare settings.

The NCPI questionnaire has also been refined for 2026, with fewer and more targeted questions aligned to the 2030 targets. It remains divided into two components: Part A, completed by national authorities, and Part B, completed by community representatives, ensuring balanced perspectives in reporting.

The lessons from past reporting cycles continue to inform the current process, providing an evidence-based roadmap for efficient implementation:

1. The national consultation process, conducted in the first quarter, facilitates timely data consolidation and validation.
2. The involvement of civil society remains critical, particularly for policy-related responses and inclusive stakeholder engagement.
3. Early engagement of data providers from January supports timely, accurate, and high-quality data submission.

The GAM reporting process for this year was spearheaded by the Strategic Information Component. The draft reports were submitted on March 31, 2026 for review and feedback.

Spectrum Report

The Spectrum software (designed by UNAIDS) is a tool used to assist countries in monitoring their HIV epidemic. It produces estimates for the number of persons living with HIV (PLHIV), the number of new infections, and other indicators used to assess epidemic trends and the overall impact of the response.

Spectrum incorporates two modelling tools for generating estimates:

1. Estimation and Projection Package (EPP):

EPP is used to estimate and project HIV prevalence trends over time and relies on observed HIV prevalence data, primarily from antenatal (ANC) clinics through sentinel surveillance and routine testing, as well as data from key populations (e.g., IBBS and other programmatic sources). This model also incorporates key population size estimates to predict the trajectory of the epidemic.

2. Case Surveillance and Vital Registration (CSAVR):

CSAVR is used when prevalence data are limited or unavailable. It estimates HIV trends based on reported new HIV/AIDS cases, AIDS-related deaths, and CD4 counts at diagnosis. Recent updates now allow comparison of multiple AIDS mortality data sources within the model to strengthen incidence estimation.

Key Updates for the reporting cycle 2025 Estimate:

- **Enhanced program inputs:** Inclusion of PrEP data for pregnant and breastfeeding women (PMTCT module) and improved validation checks for ART interruption rates to ensure realistic trends.
- **Refined assumptions:** Updated pediatric ART mortality rates and improved criteria for estimating ART impact on transmission (requiring adequate viral load coverage).
- **Improved analytical tools:** Ability within CSAVR to compare alternative AIDS mortality data sources.
- **Prevention planning:** Introduction of a Prevention Denominators Tool to estimate needs for condoms, PrEP, VMMC, and key populations using prefilled but editable national data.
- **Software access:** Spectrum is now available as both a desktop and fully functional online platform.

Model Selection

The country opted to use the EPP method for the following reasons:

- Robust programmatic data are available, including PMTCT coverage, ART coverage, and HIV testing.
- Comprehensive surveillance systems ensure reliable HIV prevalence data.
- Recent IBBS provide current HIV prevalence estimates among key populations.
- The country met all data requirements for the EPP method.

The Component submitted all relevant files for Spectrum to UNAIDS before the deadline of March 30, 2026.

STRATEGIC OBJECTIVE FOR TUBERCULOSIS

TB 1.3: Optimize TB data management, including surveillance and vital registration.

TB Database

All relevant stakeholders were consulted (laboratory, regional stakeholders, HSTU/TCS technocrats, The MOHW Surveillance Unit) in the development of the system. After a series of consultations, recommendations and training of staff, the system was completed and went live at tb.moh.gov.jm in July 2025, marking its transition into full production. Access was granted to relevant users islandwide, and security certifications were installed on their computers/laptops thus limiting access to the website.

Since implementation, both retrospective and current cases have been entered into the system, from which quarterly database reports have been generated and shared. The database includes a reporting component, where demographic and symptom reports are readily accessible.

The next steps are as follows:

- A user guide to be finalized
- Piloting of the Data Quality Audit Tool
- Adding new indicators as requested
- Creation of additional reports

A Data Quality Audit Tool has been developed and will be piloted at the National Chest Hospital to assess its functionality and applicability in the field, in preparation for full-scale data quality audits in the upcoming year.

STRATEGIC OBJECTIVE FOR SEXUALLY TRANSMITTED INFECTIONS

SO 2: Enhance Surveillance, Data Systems and Research

A series of updates have been implemented in the STI database to improve reporting accuracy and strengthen STI management. These enhancements were informed by a comprehensive database review and ongoing consultations with Contact Investigators, with targeted interventions introduced to address identified gaps. Updates to the DHIS2 platform have significantly improved both data quality and system usability.

The system now generates automated summaries, reducing the risk of calculation errors. Data entry has also been standardized through mandatory zero reporting, eliminating the use of blank fields. In addition, automated discrepancy flags have been introduced to identify and reduce missing values in real time. Complementing these changes, routine weekly and monthly data quality checks in effect will further strengthen data validation and accuracy.

Looking ahead to 2026, efforts to address under-reporting and late submissions will include extending real-time reporting by adjusting the reporting period to allow earlier data entry within the reporting month. To better differentiate late submissions from new updates, a submission cutoff period is currently being established. The database will be revised to incorporate more detailed age disaggregation for individuals aged 50 and over (50–59, 60–69, and 70+), enabling more precise of trends within the ageing population. Additionally, an STI Data Dashboard is under development within the system, which will provide a suite of visual indicators at the site, parish, regional, and national levels to support data-driven decision-making.

PRIORITIES FOR FINANCIAL YEAR (FY) 25/26

The Strategic Information Component (SI) priorities for 2026/2027 will focus on strengthening data quality, integration, analysis and use across HIV, STI, and TB programmes. Key areas include building the capacity of healthcare workers in data management, enhancing and operationalizing integrated treatment and prevention databases and ensuring timely production of routine reports such as quarterly cascade analyses and database bulletins. There will be a strong emphasis on data quality through scheduled data quality audits (DQAs) for the HIV Treatment and Prevention Databases and implementation of corrective actions, as well as the development of STI and TB audit tools.

The SI Component will also prioritize data triangulation, hotspot mapping, and the generation of strategic outputs such as the GAM report and national review meeting reports to inform decision-making. Additionally, efforts will support operational research, supportive supervision, and continuous monitoring through dashboards and action tracking mechanisms, all aimed at improving programme performance and evidence-based planning.

The priorities for the new financial year are outlined below.

Table 11: Priorities for Financial Year 26/27

Activity	Performance Measure	Due Date
Capacity building of service providers in HIV/STI/TB data management and use	Number of HCW trained in SI management	Q2-Q4
	Database training for DHIS 2 conducted by Q2	Q2
Assessment of HIV Programme Implementation	GAM report submitted by the CMO for submission to Executive Management by Q4	Q4
	Quarterly HIV cascade monitoring reports	Q2-Q4
	PLHIV (December 2025) estimated by Q1	Q1
	One hotspot map showing TB cases overlaid with risk factors	Q4
	80% of HIV/STI/TB data triangulated	Q2-Q4
	Reports from 2 national review meetings comprising the regional and CSO staff for the year.	Q2 & Q4
	3 action sheet updates from M&E Technical Working Group Meetings	Q2-Q4
	80% of actions completed on plans submitted from TSIS Quarterly Data Review Meetings	Q2-Q4
	3 HIV/STI/TB Database Bulletins created	Q2-Q4
Updated/Integrated treatment and Prevention databases	Updated/Integrated treatment and prevention databases operationalized	Q2-Q4

Activity	Performance Measure	Due Date
Management of HIV/STI/TB Databases	4 STI Database Reports submitted by Q4	Q1-Q4
	4 PMTCT Database Reports submitted by Q4	
	4 HIV Database Reports submitted by Q4	
	4 TB Database Reports submitted by Q4	
	2 quarterly Enabling Environment and Human Rights dashboard reports	Q2 & Q4
	Number of updates on published and unpublished studies/reports (including STIs and TB)	Q3 & Q3
	Updated User Manuals for the Prevention and TSIS database	Q1 & Q4
Conduct research on HIV/STI/TB	2 operational research abstracts approved	Q4
Assessment of Treatment data quality	HIV DQA conducted with at least 60% of corrective actions implemented	Q1-Q3
Assessment of Prevention data quality	Prevention DQA conducted with at least 60% of corrective actions implemented	Q1-Q3
PMTCT Data Review Meeting	4 data review meeting report produced by Q4	Q1-Q4
Development of TB Data Quality Audit Tool	TB data quality audit tool developed by Q2	Q2
Development of STI Data Quality Audit Tool	TB data quality audit tool developed by Q2	Q2
Supportive supervision visits	2 supportive supervision visits conducted as per schedule and 80% of data quality gaps addressed	Q1-Q4

APPENDICES

Operational Research 1

Prevalence of HIV Drug Resistance and Its Determinants Among Unsuppressed Persons Living with HIV in Jamaica

Ehikhamenor, C, Hanson, Y, Stewart, K, Bryan, K, Williams, A, Khan-Francis, R, Butterfield, T

Background: Jamaica has an estimated 28,000 persons living with HIV (PLHIV). The country has made steady progress in enrolling and retaining PLHIV in care on ART. While access to ART has increased over the last two decades, a growing global challenge has been the increasing prevalence of drug resistance.

Objective: The study aims to support clinical management and advance progress toward the third UNAIDS 90-90-90 target—viral suppression by describing the profile of unsuppressed PLHIV in Jamaica with HIVDR mutations.

Methods: A retrospective cross-sectional study of HIVDR samples of unsuppressed PLHIV in Jamaica was conducted between October 2024 to June 2025. A total of 212 plasma samples collected between 2018 and 2025 for HIV drug resistance (HIVDR) testing were reviewed. After clinical exclusions and laboratory quality control failures, 67 samples remained for analysis. We conducted statistical analysis using IBM SPSS version 20. Frequencies and proportions were used for categorical variables, Pearson's R test assessed associations between continuous variables, while the chi-square test examined associations between categorical variables.

Results: Of the 67 participants, 48% were male and 52% female, with a median age of 35 years. At ART initiation, 56% were classified at Stage III or IV (WHO staging). The median duration on ART at the time of HIVDR sample collection was 9 years (IQR: 7). HIVDR mutations were present in all participants. Most had NNRTI mutations (99%), followed by NRTI mutations (76%), PI mutations (28%), and INSTI mutations (12%). HIVDR mutations spanned multiple drug classes. The majority of participants demonstrated resistance to multiple drug classes: 43% (n=29) had resistance to two drug classes, 31% (n=21) to three drug classes, 22% (n=15) to a single drug class, and 3% (n=2) exhibited resistance across all four drug classes. There were no significant associations found between the number of HIVDR mutations and the following variables: viral load ($r(67) = 0.009$, $p = 0.94$), and time on ART ($r(66) = 0.1$, $p = 0.424$). However, a significant inverse correlation was observed between lower CD4 counts at ART initiation and the number of HIVDR mutations ($r(64) = -0.292$, $p = 0.019$).

Conclusion: Findings from the study indicate a high prevalence of drug resistant mutations, particularly among the Nucleotide Reverse Transcriptase Inhibitors. This resistance will likely result

in increased ART treatment failure and limited regimen options. It is imperative that the national HIV response implement more robust approaches to routine resistance testing, viral load monitoring, and other PLHIV support systems that will mitigate HIV drug resistance and maintain ART efficacy.

Operational Research 2

A Risk Assessment Matrix for Identifying Likelihood of Loss to Follow Up in Persons Living with HIV

Trought , K, Smith , J, Maragh, A, Mason, N, Chintersingh, T

Background: Within the Southern Regional Health Authority (SRHA) in Jamaica, Lost to Follow-Up (LTFU) remains a persistent challenge that undermines continuity of HIV care. Continuous engagement in treatment supports adherence to antiretroviral therapy (ART), leading to improved viral suppression and reduced morbidity and mortality. As of June 2024, SRHA recorded 2,566 patients on register, of whom 2,074 (81%) were retained in care and 492 (19%) were classified as LTFU.

Methods: To address this issue, a risk assessment matrix was developed to determine the likelihood of patients becoming LTFU by identifying and scoring predictors that negatively affect adherence. Each factor was assigned a weighted score (0–100), and an overall score was generated to categorize clients by risk level. Data were extracted from the National Treatment Site Information System (TSIS 2.0) for 2,399 eligible clients aged 18 years and older, who had been linked to SRHA treatment sites and on ART for at least five years. Statistical analyses, including Chi-Square, Phi, and Cramer’s V tests, were conducted in SPSS to examine associations between predictors and LTFU outcomes.

Results: The frequency analysis done on the data set found that the majority of clients who met the inclusion criteria were female (58.6%, n=1407) and that the highest percentage of clients were 50 years and older (36.8%, n=882). More than half the clients lived in rural areas (54.2%, n=1302) and almost a quarter of the sample had no address listed (23.3%, n=558). Majority (80%) of the eight (8) predictors contributed to the overall score statistical significance, $p < 0.001$. Patients with Low and Moderate risk scores showed very high retention (100% and 85% retained, respectively), while those in the Critical Risk category have the highest LTFU (61% lost). Gender and linkage time had no statistically significant differences. For the factors that had a Cramer’s V result, it was found that the time spent in care (0.317), and the frequency of LTFU status (0.453) had higher association strengths than a client’s age at registration (0.226) and their last viral load status (0.266).

Conclusion: These results suggest that for better retention, lower risk scores were key. There were statistically significant differences in retention in care depending on factors such as age, address type, Frequency of loss to follow up, viral load status and length of time in care. Therefore, the higher the risk score, the greater the likelihood of becoming loss to follow-up.

Operational Research 3

Mortality Patterns and Gaps in HIV Care: Insights from Jamaica, 2021–2023

Perry, D, Weller, CH, Brown, R, Gordon-Johnson, K, Douse-Mullings, A, Khan-Francis, R, Williams, A, Robb-Allen, A, Skyers, N, Beckford-Jarrett, S.

Background: In 2015, the World Health Organization recommended that PLHIV should be given Antiretroviral therapy immediately after diagnosis, which is aimed at reducing the mortality rate. Jamaica adopted the test-and-start approach in 2017. Despite these initiatives, PLHIV still experiences challenges in treatment that can affect mortality.

Objective: This study explores the demographic and clinical characteristics of persons diagnosed in Jamaica from 2021 to 2023, and how these factors influence mortality.

Methodology: We analyzed data from PLHIV 15 years and older, who were diagnosed and enrolled at treatment sites in Jamaica between January 1, 2021, and December 31, 2023, to identify how demographic and clinical characteristics influence mortality. Chi-square tests, Kaplan-Meier Survival Analysis, and Cox Regression were used to examine survival patterns and factors associated with mortality during the study period. The data was extracted on July 4, 2025.

Results: Of 3,056 individuals aged ≥ 15 years diagnosed between 2021 and 2023, 52% were male. Most were from the Southeast region, with the Northeast representing the smallest share. At baseline, 16.3% were WHO Stage 3, 18.9% Stage 4, and 10.9% had no CD4 recorded. Overall, 6.6% (n=201) died, 19.3% (n=590) were lost to follow-up, 4.5% (n=137) migrated, and 69.6% (n=2,128) remained alive and in care. Mortality was higher among males, those aged ≥ 60 years, persons with Stage 4 CD4 (<200), or missing baseline CD4, and those diagnosed in 2021. Independent predictors of death included age 25–59 years (AHR 4.72), ≥ 60 years (AHR 10.05), Stage 4 CD4 (AHR 2.38), and no CD4 recorded (AHR 8.81).

Conclusion: Mortality outcomes have improved but may be underestimated due to loss to follow-up. Deaths were concentrated among older clients and those with advanced disease or missing CD4 data, underscoring gaps in testing, documentation, and retention. Strengthening early diagnosis, ART initiation, and CD4 monitoring remains critical to improving survival.

Operational Research 4

Retention in Care Among PLHIV Accessing Care at Public Treatment Sites in Jamaica – S01

Lewis, Selena, Khan-Francis, Rahanah, Skyers, Nicola, Brissett, Shanille, Whitehead, Akeem, Williams, Andre

Objectives: To characterize Jamaica's HIV population based on retention status, identify factors associated with retention in care, and develop recommendations to improve retention.

Methods: A retrospective analysis was conducted using data from the Treatment Site Information System (TSIS). A total of 40,272 PLHIV were identified, with exclusions for duplicates, transfers, non-regional health authority attendees, and deaths, leaving 12,542 for analysis. Retention was defined as active HIV care engagement without missed appointments >90 days, while loss to follow-up (LTFU) was >90 days without a clinic visit. Retention status was compared across demographic, socioeconomic, and clinical variables using descriptive statistics and logistic regression to identify factors significantly associated with retention.

Results: Overall, 83% (n=10,466) were retained and 17% (n=2,076) were LTFU. Retention was significantly higher among females (84% vs. 82%, $p=0.003$) and married individuals (OR=2.0, $p<0.001$). Retention was lowest in PLHIV aged 20–44 years compared to those ≥ 50 ($p<0.001$). PLHIV receiving care outside their parish of residence were less likely to be retained (OR=0.7, $p<0.001$). Receiving SMS reminders was strongly associated with retention (OR=20.1, $p<0.001$). Clinical factors associated with higher retention included first CD4 <200 (OR=1.9, $p<0.001$) and viral load suppression, with undetectable VL increasing retention fivefold (OR=5.4, $p<0.001$).

Conclusion: Retention in HIV care at public sites in Jamaica is generally high but suboptimal among younger age groups, males, and those accessing care outside their parish. Strengthening SMS reminder systems, targeted interventions for vulnerable groups, and expanded service access are recommended to improve retention.